

Zepbound

Brand-Name GLP-1 · Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v1.2 · generated 2026-08-09

Zepbound is written as a Tebra STANDARD prescription, never Tebra Compound. KORB does not manage fulfillment, does not complete prior authorizations, and does not handle coupons.

Available states

TX, CA, AL, WI

Brand-name GLP-1 through the manufacturer programs is only available in Texas, California, Alabama and Wisconsin. Source: the brand prescribing document. This is a program limit, not a shipping limit — do not offer a brand pathway to a patient outside these four states.

Rules and visit fee

Order via	Tebra Standard prescription (NOT Tebra Compound)
Billing	Patient pays the manufacturer program or the local pharmacy directly.
Prior authorizations	KORB does NOT complete insurance prior authorizations.
Coupons and savings cards	KORB does NOT handle coupons or savings cards.
Patient responsibility	KORB does not complete insurance prior authorizations and does not handle coupons or manufacturer savings cards. The patient is responsible for checking pricing, coverage and every other aspect of the medication.
Local pharmacy	A brand prescription can be sent to the patient's local pharmacy on request, under the same rules. KORB will not re-send a prescription between pharmacies to chase a lower price. The patient does that comparison before ordering.
Prescription visit fee	\$79 — charge code FITGLP1001
Fee covers	Cash-pay only. The fee covers the clinical visit, prescription issuance and program administration. Medication cost, pharmacy processing, shipping and supplies are set by the external pharmacy and paid directly by the patient.
Non-refundable	The visit fee is non-refundable once the visit has occurred or a prescription has been issued. Medication cost is not included.

Medication pricing is deliberately not listed

Manufacturer pricing changes without notice and KORB does not control it. Storing it creates a maintenance burden and risks providers quoting incorrect pricing to patients. Price and coverage are the patient's responsibility.

Zepbound — KwikPen

Route: subcutaneous • **Frequency:** once weekly

Order via: Tebra Standard prescription

Pharmacy: LillyDirect Self Pay Pharmacy Solutions, 4343 Equity Dr, Columbus, OH 43228-3842 • (833) 432-4322

KwikPen only. Do NOT prescribe the single-dose vial presentation.

One KwikPen holds 4 fixed weekly doses. The pen locks out after the fourth dose and is then discarded, including any leftover medicine. Do not transfer from the pen into a syringe.

Two presentations in the dropdown — pick carefully

Tebra lists BOTH presentations. The vial reads "Zepbound X mg/0.5 ml subcutaneous solution"; the pen reads "Zepbound kwikpen X mg/0.6 ml ...". Selecting the 0.5 mL vial with quantity 1 dispenses ONE WEEK instead of four. The bracketed figure in the pen entry is total pen content — 4 doses x 0.6 mL = 2.4 mL — which is Tebra confirming the 4-dose device.

Dispensing option	Quantity	Refill	Days	When to use
4-week (28-day)	1	0	28	New prescription or any dose change. Use until the dose is stable.
8-week (56-day) — 1 pen + 1 refill	1	1	28	Clinically stable patients only. Each fill is one pen covering 4 weeks.

Tebra STANDARD fields — 4-week (28-day)

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
2.5 mg	Zepbound kwikpen 2.5 mg/0.6 ml (10 mg/2.4 ml) subcutaneous pen injector	Inject 2.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	0	28
5 mg	Zepbound kwikpen 5 mg/0.6 ml (20 mg/2.4 ml) subcutaneous pen injector	Inject 5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	0	28
7.5 mg	Zepbound kwikpen 7.5 mg/0.6 ml (30 mg/2.4 ml) subcutaneous pen injector	Inject 7.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	0	28
10 mg	Zepbound kwikpen 10 mg/0.6 ml (40 mg/2.4 ml) subcutaneous pen injector	Inject 10 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	0	28
12.5 mg	Zepbound kwikpen 12.5 mg/0.6 ml (50 mg/2.4 ml) subcutaneous pen injector	Inject 12.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	0	28
15 mg	Zepbound kwikpen 15 mg/0.6 ml (60 mg/2.4 ml) subcutaneous pen injector	Inject 15 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	0	28

Tebra STANDARD fields — 8-week (56-day) — 1 pen + 1 refill

Dose	Drug (dropdown)	Patient Instructions	Qty	RF	Days
2.5 mg	Zepbound kwikpen 2.5 mg/0.6 ml (10 mg/2.4 ml) subcutaneous pen injector	Inject 2.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	1	28
5 mg	Zepbound kwikpen 5 mg/0.6 ml (20 mg/2.4 ml) subcutaneous pen injector	Inject 5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	1	28
7.5 mg	Zepbound kwikpen 7.5 mg/0.6 ml (30 mg/2.4 ml) subcutaneous pen injector	Inject 7.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	1	28
10 mg	Zepbound kwikpen 10 mg/0.6 ml (40 mg/2.4 ml) subcutaneous pen injector	Inject 10 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	1	28
12.5 mg	Zepbound kwikpen 12.5 mg/0.6 ml (50 mg/2.4 ml) subcutaneous pen injector	Inject 12.5 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	1	28
15 mg	Zepbound kwikpen 15 mg/0.6 ml (60 mg/2.4 ml) subcutaneous pen injector	Inject 15 mg subcutaneously once weekly for 4 weeks. Multi-dose pen – discard after 4 doses.	1	1	28

- Sent electronically through Tebra as a STANDARD prescription. No insurance billing and no prior authorization.
- Patient completes enrolment, payment and shipping directly with LillyDirect.
- PEN NEEDLES ARE REQUIRED and are not included. The patient purchases them, and they can be added at LillyDirect checkout. Each injection uses a new needle.
- Zepbound is sold in three formats. KORB uses the KwikPen only.
- Rotate injection sites. Follow standard tirzepatide titration principles.
- Early refills or replacement shipments are decided by the pharmacy, not KORB.

Is this the right patient

Program: **Metabolic Health & Weight Loss Program** · Visit cadence: **Every 4 to 8 weeks**
Indications

- Weight loss
- Medically driven weight loss
- Increased energy
- Better sleep
- Improved mental health
- Improved cardiovascular health

Identifying the appropriate candidate

Body mass index	BMI greater than 25 is overweight · BMI greater than 30 is obese
FDA-approved uses	• Weight loss • Type 2 diabetes • Prevention of cardiovascular death, myocardial infarction and stroke in adults with established cardiovascular disease who are overweight or obese
Diabetic and pre-diabetic patients	KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Agent monograph — Tirzepatide — dual GIP and GLP-1 receptor agonist

Definition	A 39-amino-acid synthetic peptide based on the native GIP sequence, engineered to agonise both the glucose-dependent insulinotropic polypeptide (GIP) receptor and the GLP-1 receptor. A C20 fatty diacid moiety extends the half-life to roughly five days, allowing once-weekly dosing. FDA-approved as Mounjaro for type 2 diabetes and as Zepbound for chronic weight management and obstructive sleep apnoea in adults with obesity.
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Mechanism

- Dual incretin agonism — both the GIP and the GLP-1 receptor.
- GLP-1 component: glucose-dependent insulin secretion, glucagon suppression, delayed gastric emptying, central appetite reduction.
- GIP component: enhanced insulin secretion and improved insulin sensitivity in adipose tissue.
- GIP agonism may also blunt the nausea seen with GLP-1 activity alone, one proposed reason tolerability holds up at higher relative efficacy.

Clinical evidence

- SURMOUNT-1: mean weight loss around 20% of body weight at 15 mg over 72 weeks — the largest effect seen with a pharmacological agent in this class.
- SURPASS programme: glycaemic efficacy in type 2 diabetes, generally exceeding semaglutide in head-to-head comparison.
- SURMOUNT-OSA: supported the obstructive sleep apnoea indication.
- As with semaglutide, weight regain after discontinuation is well documented.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Contraindications

Absolute — do not initiate	Cautions — evaluate before initiating
• Personal or family history of medullary thyroid carcinoma (MTC)	• History of pancreatitis — evaluate carefully before initiating
• Multiple endocrine neoplasia syndrome type 2 (MEN2)	• Gastroparesis or other significant gastrointestinal motility disorder
• Known hypersensitivity to tirzepatide or any component of the formulation	• Active gallbladder disease or prior gallbladder-related surgical complications
• Pregnancy, breastfeeding, or planning pregnancy	• Severe renal impairment (eGFR below 30 mL/min/1.73 m ²)
	• Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
	• History of suicidal ideation or active depression — monitor and ask

Medication interactions

- ORAL CONTRACEPTIVES: tirzepatide reduces the effectiveness of oral hormonal contraception. Advise a non-oral method, or add a barrier method, for four weeks after initiation and for four weeks after each dose increase. This is specific to tirzepatide and is easy to miss.
- Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
- Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
- Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
- ANAESTHESIA AND PROCEDURES: delayed gastric emptying raises aspiration risk under sedation. Anaesthesia guidance generally advises holding weekly GLP-1 therapy before an elective procedure. Tell the patient to disclose use to any surgeon, proceduralist or anaesthetist.
- Other GLP-1 or dual incretin agonists: do not combine.

Side effects

Common	Less common
• Pain, redness or swelling at the injection site	• Pancreatitis
• Nausea and vomiting	• Gastroparesis
• Diarrhea	• Bowel obstruction
• Stomach pain	• Gallstone attacks
• Constipation	
• Low appetite	
• Headaches	
• Dizziness	

Monitoring

■ Weight and BMI at each visit	■ Right-upper-quadrant pain — assess for gallbladder disease
■ Ask about blood pressure — monitored by the PCP, not measured by KORB	■ Contraceptive method if the patient uses oral hormonal contraception
■ Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB	■ Mood and mental health
■ Gastrointestinal tolerance, especially through dose escalation	■ Lean mass preservation — protein intake and resistance training
■ Abdominal pain that is severe or radiates to the back — assess for pancreatitis	■ Injection site and adherence

Patient counseling

“Tirzepatide works on two gut hormone pathways rather than one, which is why it tends to produce more weight loss than semaglutide. It slows stomach emptying and reduces appetite. Nausea is the most common side effect and is usually worst in the days after a dose increase. This is a compounded preparation rather than the brand-name product. If you take a birth control pill, it may not work as reliably for four weeks after we start and after each dose increase, so use a backup method. Weight tends to come back if the medication stops. Tell any surgeon or anaesthetist that you are taking this.”

- Gastrointestinal side effects are most common during dose escalation.
- Slower titration or holding at the current dose may improve tolerability.
- Instruct the patient to report persistent, severe or worsening symptoms.
- New or severe symptoms may require dose adjustment, temporary hold, or discontinuation.
- Vials are good for 28 days from first use regardless of the pharmacy BUD. Counsel the patient to discard at 28 days even if medication remains.

Chart attestation

Patient counseled on tirzepatide as a dual GIP/GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, reduced oral contraceptive effectiveness with the recommended backup precautions, the need to disclose use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

Primary	Secondary
E66.9 — Obesity, unspecified	Z68.— — Body mass index (add the matching BMI code)
E66.01 — Morbid (severe) obesity due to excess calories	G47.33 — Obstructive sleep apnea (if documented and relevant)

Primary	Secondary
E66.3 — Overweight	Z71.3 — Dietary counseling and surveillance
	Z72.3 — Lack of physical exercise

Escalation and discontinuation

Hard stop — discontinue immediately	Flag and reassess
• Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.	• Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
• Hypersensitivity or anaphylaxis-type reaction.	• Right-upper-quadrant pain, or gallstone symptoms.
• Pregnancy identified or confirmed.	• Rapid or excessive weight loss, or loss of lean mass.
• Medullary thyroid carcinoma or MEN2 identified at any point.	• New or worsening depression, or any mention of suicidal thoughts.
• Persistent severe vomiting with dehydration or acute kidney injury.	• Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.
	• Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
	• Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

• GLP-1 receptor agonists are not currently on the WADA prohibited list, so the athlete hard stop used in the Functional Health peptide program does not apply to this program. Confirm current WADA status before treating a tested competitive athlete.

Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Operations
- Clinical protocol questions — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute a pharmacy the tool marks as unavailable
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without confirming with Operations